A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

27 May 2026

Mr Michael Johnston
Melbourne Colorectal Specialists

Dear Mr Johnston,

Re: Mrs Suzanne Matheson
DOB: 04/12/1964 Phone: 0422 310 175
Address: 38/ 56 Beach Road, HAMPTON, VIC, 3188
Email: mathesonsue19@gmail.com

Summary

- **Bowel urgency and loose stools following an episode of diverticulitis.**
- **Likely post-inflammatory visceral hypersensitivity and altered motility, with potential bile acid malabsorption contributing.**
- **Commenced low-dose nortriptyline, psyllium husks, and lifestyle modifications.**
- **No action required by you**

I had the pleasure of conducting an in-person consultation with Suzanne Matheson, who was referred by Dr Michael Johnson regarding a change in bowel habit.

Symptoms and history of presenting complaint

Suzanne's symptoms date back to early May 2024, following a five-day hospital admission for her first episode of diverticulitis, which was managed with intravenous antibiotics. Since that time, she has experienced significant bowel urgency and a "fast" transit time.

She typically opens her bowels twice daily, though on symptomatic days this increases to four times. Her stools are often loose, frequently occurring in an "explosive" fashion after an initial firmer motion. She reports a sense of incomplete evacuation approximately 70% of the time. She suffers from spasmodic abdominal pain about twice a week, which is generally associated with her bowel movements. These symptoms have caused her significant distress and anxiety, particularly when travelling or hiking, often requiring the prophylactic use of loperamide for confidence.

Diet

Suzanne has already made several sensible modifications, including transitioning to decaffeinated coffee and tea. She currently uses a "gentle fibre" supplement with her breakfast. She continues to eat high-FODMAP foods, including apples and curries containing onion and garlic. She also occasionally uses mints and chewing gum.

Past medical history and medications

Suzanne's history is notable for a cholecystectomy performed approximately 10 years ago. She has a single functioning kidney following a childhood ureteroplasty and a subsequent nephrectomy 25 years ago. She has

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also undergone bilateral big toe arthrodeses for arthritis and has a history of excised melanomas and BCCs. She is post-menopausal and recently trialled HRT but ceased this due to breakthrough bleeding. She currently takes no regular prescription medications.

Social history, family history

A primary school teacher, Suzanne lives with her husband in Kensington. There is a strong family history of diverticular disease; both of her brothers have required bowel resections for diverticulitis. Her mother also suffered from similar bowel urgency later in life.

Examination

On examination, Suzanne appeared well. Her abdomen was soft and non-tender, with no palpable masses or organomegaly. I noted well-healed surgical scars consistent with her previous nephrectomy and cholecystectomy.

Impression and plan

I suspect Suzanne is suffering from post-inflammatory irritable bowel syndrome (PI-IBS) following her diverticulitis earlier this year. This has likely resulted in visceral hypersensitivity and altered colonic contractility. Her previous cholecystectomy may also be a contributing factor, as the continuous flow of bile into the duodenum can act as a secretagogue, exacerbating loose stools and urgency.

My plan is as follows:

- **Investigations:** I have requested a screen of baseline bloods and stool studies (including calprotectin) to exclude ongoing inflammation or malabsorption.
- **Neuromodulation:** I have commenced Suzanne on a low dose of nortriptyline (10 mg at night). I explained that at this sub-therapeutic dose, the medication acts on the gut-brain axis to reduce hypersensitivity and slow colonic transit. She will titrate this to 20 mg after two weeks if tolerated.
- **Fibre and Supplements:** We will increase her psyllium husk intake to two teaspoons daily to help "bulk" the stool and improve evacuation.
- **Diet and Lifestyle:** I have advised her to avoid apples, pears, mints, and chewing gum (which contains sorbitol) for the time being. I have also recommended the use of a footstool to optimise her anorectal angle during defaecation to assist with complete emptying.

I will review Suzanne in eight weeks to assess her response to these measures.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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CC: Dr Andrew J Harris

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